Clinical Study Summary Report (CSSR) Document Status: Final | **Version:** 1.0 | **Date:** 03-Apr-2026

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1. Administrative & Study Identification

Full Study Title: A Phase 2, Randomized, Double-Blind, Placebo-controlled Study to Compare Efficacy and Safety of Oral Azacitidine Plus Best Supportive Care Versus Best Supportive Care as Maintenance Therapy in Japanese Subjects With Acute Myeloid Leukemia in Complete Remission **Short Title/Acronym:** Oral Azacitidine and Best Supportive Care for Japanese AML **Protocol Number:** CA055-005 **NCT Number:** NCT05197426 **Sponsor Name:** Bristol Myers Squibb **Investigational Product:** Oral Azacitidine (CC-486) / Placebo **Phase of Development:** Phase 2 **Medical Monitor:** Bristol Myers Squibb Clinical Operations Lead

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the efficacy of oral azacitidine plus best supportive care versus best supportive care (placebo) as maintenance therapy in a cohort of Japanese participants ≥ 55 years of age with Acute Myeloid Leukemia (AML) in complete remission. **Secondary Objectives:** To evaluate Overall Survival (OS), Time to Relapse, Health-Related Quality of Life (assessed via EQ-5D-5L and FACIT-Fatigue scales), and Pharmacokinetics (Cmax, AUC).

2.2 Background & Rationale To address the need for effective, tolerable maintenance therapies for older Japanese patients with Acute Myeloid Leukemia (AML). Following intensive induction chemotherapy, patients who achieve complete remission (CR) or complete remission with incomplete blood count recovery (CRi) remain at high risk of relapse. Oral azacitidine represents a feasible maintenance treatment to prolong relapse-free survival and overall survival in this population, preventing or delaying disease recurrence while aiming to preserve quality of life.

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3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** Randomized

3.2 Planned Enrollment & Duration Trial Status: Active, not recruiting **Target N:** 19 (Actual Enrollment) **Number of Sites:** Multiple sites in Japan **Estimated Study Start:** 17-Jan-2022 **Estimated Primary Completion:** 27-Jan-2025 (Actual) **Estimated Study Completion:** 30-Apr-2026

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. ≥ 55 years of age inclusive at the time of signing the informed consent. 2. Newly diagnosed, histologically confirmed de novo Acute Myeloid Leukemia (AML) or AML secondary to prior myelodysplastic syndrome (MDS) or chronic myelomonocytic leukemia (CMML). 3. Achieved complete remission (CR) or complete remission with incomplete blood count recovery (CRi) after conventional induction chemotherapy with or without consolidation chemotherapy.

4.2 Exclusion Criteria 1. Suspected or proven acute promyelocytic leukemia; or AML with previous hematologic disorder such as chronic myeloid leukemia or myeloproliferative neoplasms (excluding MDS and CMML). 2. Prior bone marrow or stem cell transplantation. 3. Received therapy with hypomethylating agents for MDS and went on to develop AML within four months of discontinuing the therapy; or achieved CR/CRi following therapy with hypomethylating agents.

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Oral Azacitidine (CC-486; Drug Name: azacitidine; Trade Names: Vidaza, Onureg; ATC Code: L01BC07) or matching Placebo administered in treatment cycles. **Route of Administration:** Oral **Duration of Treatment:** Continued until documented disease relapse, unacceptable toxicity, dropout, or study termination.

5.2 Concomitant & Prohibited Therapy Permitted: Best Supportive Care (BSC) to manage symptoms and cytopenias. **Prohibited:** Other antineoplastic agents, commercial hypomethylating agents, and unapproved investigational drugs.

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6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Leukemia baseline assessment (CR/CRi confirmation)
Baseline	Day 0	Randomization, First Dose, Vital Signs, QoL Questionnaires
Treatment Cycles	Varies	IP Administration, Pharmacokinetics (PK) sampling, Safety Labs, Efficacy Assessment, EQ-5D-5L/FACIT-Fatigue
End of Treatment	At Discontinuation	Final Vital Signs, Exit Interview, IP Accountability, Response Assessment
Follow-up	Post-treatment	Survival Follow-up, Relapse monitoring

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Relapse-Free Survival (RFS): Time from randomization to the date of documented relapse after CR or CRi per central review, or death from any cause, whichever occurs first [Time Frame: Up to study completion, ~4 years]. **Measurement Method:** Clinical and hematological evaluation defined according to the IWG AML response criteria.

7.2 Secondary & Exploratory Endpoints 1. Overall Survival (OS): Time from randomization to death from any cause. 2. Time to Relapse: Interval from the date of randomization to the date of documented relapse. 3. Health-Related Quality of Life: Measured by EQ-5D-5L health utility index and FACIT-Fatigue Scale. 4. Pharmacokinetics: Maximum plasma concentration (Cmax) and Area under the plasma concentration time-curve (AUC).

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous collection from the time of informed consent through the end of the follow-up period. **Serious Adverse Events (SAEs):** Must be reported to the sponsor within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** Utilized for ongoing evaluation of subject safety and to monitor double-blind efficacy data.

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9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for all primary efficacy analyses. Safety Set for toxicity evaluations. **Sample Size Justification:** Sized to provide descriptive estimates of efficacy (RFS) and safety for the targeted Japanese demographic. **Handling of Missing Data:** For RFS, participants who are alive without documented relapse are censored at the date of their last response assessment. A competing risk analysis is utilized for Time to Relapse where death without documented relapse is treated as a competing risk.

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and centralized monitoring visits to ensure protocol adherence and data validity. **Audit Trail:** Maintained through an electronic Data Capture (eDC) system with eCRF locking and comprehensive data clarification processes.

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11. Study Identifiers & Governance

Primary ID: CA055-005 **NCT Number:** NCT05197426 **Posting ID:** 1038523475209203280 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** A Study to Compare the Efficacy and Safety of Oral Azacitidine Plus Best Supportive Care Versus Best Supportive Care as Maintenance Therapy in Japanese Participants With Acute Myeloid Leukemia (AML) in Complete Remission **Official Regulatory Title:** A Phase 2, Randomized, Double-Blind, Placebo-controlled Study to Compare Efficacy and Safety of Oral Azacitidine Plus Best Supportive Care Versus Best Supportive Care as Maintenance Therapy in Japanese Subjects With Acute Myeloid Leukemia in Complete Remission

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12. Clinical Framework (AML Specific)

Primary Condition: Acute Myeloid Leukemia (AML) **Preferred Disease Name:** Leukemia, Myelocytic, Acute **Diagnosis Threshold:** Morphologic/Histologic confirmation of de novo or secondary AML **Medical Methodology:** Post-induction Maintenance Therapy **Optimization Target:** Relapse-Free Survival (RFS) prolongation in older patients (≥ 55 years)

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13. Study Procedures & Methodology

Management Pathway: Maintenance setting follow-up post-CR/CRi **Education Protocol (HCP):** Site initiation and continuous monitoring on dosing and toxicity management guidelines **Education Protocol (Patient):** Counseling on compliance to oral administration, Best Supportive Care, and QoL instrument completion **Quality Audit Mechanism:** Centralized review of relapse documentation and ongoing blinded data review

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14. Observation & Follow-Up Schedule

Total Duration: Up to roughly 48 months (until study completion) **Onsite Visit Cadence:** Pre-defined day 1 of each treatment cycle and intermediate safety visits **Remote Monitoring Frequency:** Routine data quality reviews **Checklist Review Interval:** Routine continuous assessment of OS/RFS status

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15. Sign-off & Approval

| Role | Name | Signature | Date | | :--- | :--- | :--- | :--- | | **Principal Investigator** | [Name] | ____ **| [DD-MMM-YYYY] | | Clinical Operations Lead | [Name] | ___ |** ***[DD-MMM-YYYY]*** **| |** ***Lead Statistician*** **|** ***[Name]*** **|** _____ | [DD-MMM-YYYY] |